

PRINTABLE DESK KIT

The Printable PT Desk Kit

15 Quick References · 5 Patient Handouts · 3
Burnout Mini-Tools · 1 Custom Template

Print these pages. Laminate the ones you use daily.
Tape them to your desk. Keep a copy in your bag.
These are the sheets you reach for when you need a
script fast.

FROM THE PT COMMUNICATION RESOURCE

QUICK REFERENCE 1

"When a Patient Says X, Try Y"

PATIENT SAYS	TRY SAYING
"I didn't have time."	"That is real. What is one small thing you could fit in, even on a busy day?"
"It hurts."	"Show me where. Let us find a version that does NOT hurt."
"I will do it later."	"Later rarely comes. What is the smallest thing you could do right now?"
"I don't feel ready."	"You may never feel ready. Can we try one small step and see how it feels?"
"This is not working."	"Tell me more. What were you expecting that is not happening?"

"I am fine."

"I am glad to hear that. Let me show you what 'fine' looks like in a strength test — sometimes we have hidden gaps."

"I already tried that."

"Good to know. Can you tell me what happened? That way we do not repeat what did not work."

"My friend said..."

"Every body is different. What worked for your friend might not work for you — and vice versa."

"I am just getting old."

"Aging changes things. But it does not sentence you to pain. We can still make progress."

"I don't want to be a burden."

"You are not a burden. Asking questions helps me help you. That is literally my job."

QUICK REFERENCE 2

Top 15 Phrases Patients Misunderstand

"Take it easy."

| What patients hear: "Do nothing."

| Better: "Do a little less than you think you can."

"Listen to your body."

| What patients hear: "Stop if it hurts at all."

| Better: "Pain is information. Tell me what you feel, and we will decide together."

"You will get there."

| What patients hear: "Not today."

| Better: "Here is what 'there' looks like. Here is where you are now. Here is the bridge."

"Consistency matters."

| What patients hear: "Do it perfectly every day."

| Better: "Showing up counts even on the bad days."

"Rest is important."

| What patients hear: "Rest is lazy."

| Better: "Rest is when your body repairs. It is part of the plan."

"Do not overdo it."

| What patients hear: "Do not push at all."

| Better: "Find the edge. Stop before you fall off it."

"Patience."

| What patients hear: "Stop complaining."

| Better: "This stage is frustrating. That is normal. Let me show you why it matters."

"Everyone is different."

| What patients hear: "I have no answer."

| Better: "Your body has its own timeline. Here is what we can control."

"Trust the process."

| What patients hear: "Trust me blindly."

| Better: "Here is the evidence this works. Let us track your progress together."

"You are doing great."

| What patients hear: "You are done."

| Better: "You have come far. Here is what is next."

"Modify as needed."

| What patients hear: "Do whatever you want."

| Better: "Here is the safe range. Modify within these boundaries."

"It might get worse before better."

| What patients hear: "Expect more pain."

| Better: "Some people notice temporary soreness. That is normal if it settles."

"Quality over quantity."

| What patients hear: "Do fewer reps."

| Better: "Doing these perfectly matters more than doing many poorly."

"Stay positive."

| What patients hear: "Ignore your pain."

| Better: "Your mindset matters — but so does your pain. Let us hold both."

"You have to want it."

| What patients hear: "If not better, it is your fault."

| Better: "Effort matters. But your body has its own timeline too."

QUICK REFERENCE 3

First Visit Script (Setting Expectations)

5-Minute Script for the First Visit

"Here is how I work. I am going to teach you things. You are going to practice at home. The people who get the best results are the ones who show up — even on days they do not feel like it.

Recovery is not a straight line. You will have good weeks and hard weeks. That is normal. When a hard week happens, do not quit. Tell me. We adjust.

My job is to give you the right plan. Your job is to do your best with it. If something is not working, that is my fault until we fix it together.

Any questions before we start?"

Printable Version for New Patients

Keep this card. Read it again when you are frustrated.

Recovery has ups and downs. Setbacks
are not starting over.

Tell me when something is not working. I
will adjust.

Showing up on bad days counts more
than perfection on good days.

You are not a burden. Asking questions
helps me help you.

QUICK REFERENCE 4

"I'm Fine" Visit Script (Early Discharge)

"I am glad you are feeling better. That means phase one worked.

But there is a difference between feeling better and being stronger. Feeling better can vanish in a week. Being stronger stays with you.

Here is what I want you to consider: let us finish the full plan. Not because you need to feel better — because you want to stay better.

If you still want to discharge today, I will support that. But I want you to know what you are leaving behind. Let me show you one thing first."

If They Discharged Early and Relapsed:

"This happened because we stopped too soon. That is not your fault. I should have been clearer. Now we know. Let us finish the full plan this time. No shortcuts."

QUICK REFERENCE 5

Frustrated Patient Script (Plateau/Setback)

"I hear your frustration. You were moving forward, and now you feel stuck. That is hard.

Here is what I know: plateaus are normal. They are not a sign of failure. They are a sign that your body needs something different.

So let us do something different. I am going to adjust the plan. Not because you did anything wrong — because that is my job.

Can we agree to try this new plan for two weeks before we decide if it is working?"

For the Patient Who Wants to Quit:

"I understand why you want to quit. But let me ask you: if you quit now, what happens in six months? Does this problem go away, or does it come back worse? Let us try one more adjustment before we give up."

QUICK REFERENCE 6

Non-Adherent Patient Script (Gentle)

"I have noticed you have been having a hard time keeping up with the home exercises. That happens. It is not a moral failure.

Let me ask you: is the problem motivation, time, pain, or something else? Be honest. I have heard it all.

Once I know the real barrier, we can solve it together. Smaller plan. Different exercises. A reminder system. Whatever you need.

The only wrong answer is pretending everything is fine when it is not. Help me help you."

If They Admit They Haven't Been Trying:

"Thank you for being honest. That is brave. Now let us fix it — together. What is the smallest, easiest version of this plan that you could actually do?"

QUICK REFERENCE 7

Red Flags vs. Yellow Flags (When to Refer Out)

RED FLAG (STOP. REFER IMMEDIATELY.)	YELLOW FLAG (PROCEED WITH CAUTION.)
Sudden, severe, unexplained pain	Gradual worsening over weeks
Loss of bladder or bowel control	Mild morning stiffness
Progressive weakness in limbs	Fatigue that improves with rest

Chest pain or shortness of breath	History of similar episodes
Fever with unexplained musculoskeletal pain	Stress or mood affecting symptoms
Night pain that wakes patient from sleep	Mild swelling without redness or heat
Unexplained weight loss	Deconditioning from inactivity
Trauma with possible fracture	Patient frustration or low motivation

Red Flag Script: *"I am concerned. This is outside what I can safely treat. You need to see your doctor — today if possible."*

Yellow Flag Script: *"This is not an emergency, but we need to be careful. Let us start very conservatively."*

QUICK REFERENCE 8

The 5-Minute Morning Check-In for PTs

For you, not for patients. Do this before your first patient.

1 min — Body scan: Am I carrying tension? Did I sleep enough? Am I hungry or thirsty? Fix it now.

1 min — Emotional check: How am I feeling? Am I carrying anything from yesterday? Name it. Set it down.

1 min — Intentions: What is one thing I want to do well today? Who is the one patient I need to be most present for?

1 min — Boundaries: What is one request I will say no to? What is one thing I will protect (lunch, documentation time, leaving on time)?

1 min — Quiet shift: Three slow breaths. "I am here. I am enough. I will help who I can."

QUICK REFERENCE 9

Patient Personality Types and How to Talk to Each

The Skeptic

Needs: Evidence, logic, proof

Say: "Here is what research shows..."

Avoid: "Trust me" or emotional appeals

The Pleaser

Needs: Permission to be honest

Say: "I need your real answer, not the nice answer."

Avoid: Assuming they are fine because they say so

The Overwhelmed

Needs: Simplicity, small steps

Say: "Let us focus on just one thing this week."

Avoid: Information overload

The Competitive

Needs: Metrics, comparison to self

Say: "Last week you did X. This week you did Y."

Avoid: Comparing them to others

The Victim

Needs: Empowerment, choice

Say: "You have options. Here are three. Which one feels right?"

Avoid: Pity or rescuing

The Impatient

Needs: Timeline, milestones

Say: "Here is the roadmap. Here is where you are. Here is what is next."

Avoid: "Be patient" or vague reassurance

The Denier

Needs: Gentle reality testing

Say: "Help me understand what you think is happening versus what I am seeing."

Avoid: Confrontation or "you are in denial"

The Dependent

Needs: Confidence, self-efficacy

Say: "You did that on your own. I just watched."

Avoid: Creating reliance on you

QUICK REFERENCE 10

The 10 Most Common Patient Fears (and What to Say)

FEAR	WHAT TO SAY
"I will make it worse."	"That fear is common. Let us find a version that does NOT make it worse."
"I am too old."	"Aging changes things but does not sentence you to pain. We can still make progress."
"I have tried everything."	"You have tried a lot. But not everything. Let me show you one approach you might not have seen."
"I do not have time."	"What if the plan took 10 minutes? Could you find 10 minutes?"
"It will never get better."	"I hear that. That feeling is real. But it is not the same as a fact. Let me show you why."
"I am afraid of surgery."	"Most conditions we treat do NOT end in surgery. Here is what the research says."

"I am letting my family down."

"Your family wants you better, not perfect. Showing up here is already helping them."

"I do not trust my body."

"Your body is trying to protect you. Sometimes it overprotects. Let us teach it a new way."

"I am in too much pain."

"Pain is loud. I get it. Let us start with something so small it barely registers."

"What if nothing works?"

"That is a scary thought. But we are not guessing — we have a plan. And if this plan does not work, we adjust."

QUICK REFERENCE 11

Motivational Interviewing Cheat Sheet for PTs

The OARS Framework: Open questions, Affirmations, Reflective listening, Summarizing

Open-Ended Questions

"What matters most to you about getting better?"

"How would your life be different if this problem was gone?"

"What have you tried so far, and what happened?"

Affirmations

"You showed up today. That counts."

"It took courage to say that."

"You are trying hard. I see that."

Reflective Listening

"It sounds like you are frustrated because you expected faster progress."

"What I hear is that you want to get better but you are scared of making it worse."

Summarizing

"Let me make sure I understand. You want to get back to running, but every time you try, your knee swells. You are wondering if it is even possible. Did I get that right?"

Questions That Open Doors

"What would you be doing right now if pain was not in the way?"

"On a scale of 0-10, how important is getting better to you?"

"On a scale of 0-10, how confident are you that you can do this?"

"What is one thing that would make this easier for you?"

"If you decided to try, what would that look like?"

QUICK REFERENCE 12

Pain Education Talking Points

1. Hurt Does Not Equal Harm

Pain during exercise is common. It does not mean tissue damage. Soreness that settles within 24 hours is usually safe.

"Hurt does not always mean harm. Like touching a hot stove hurts, but so does a tight muscle. One is dangerous. One is just uncomfortable."

2. The Alarm System

Your nervous system acts like an alarm. After injury, the alarm becomes oversensitive. It rings when nothing is actually wrong.

"Your body's alarm system is overreacting. It is like a car alarm that goes off when a cat walks by. We need to recalibrate it."

3. The Boiled Frog

Avoidance makes sensitivity worse. Gradual exposure retrains the alarm system.

"If you avoid everything that hurts, your world gets smaller and smaller. We need to gently expand it again."

4. The Load Dial

Tissue needs load to heal. Too little = weakness. Too much = irritation. The right amount = adaptation.

"Think of it like a dial. We are turning the load up just enough to challenge your body, not enough to overwhelm it."

5. Time vs. Tissue

Pain that persists beyond normal healing time is more about sensitivity than ongoing damage.

"Your tissue has healed. The pain you feel now is your nervous system, not damaged tissue. That is actually good news — it means we can retrain it."

QUICK REFERENCE 13

The HEP Prescription Quick Sheet

Starting Point

VARIABLE	RECOMMENDATION
Number of exercises	2-4
Frequency	3-5 days per week
Sets	1-2
Reps	8-12
Hold time (static)	10-30 seconds

Rest between sets

30-60 seconds

When to Progress

2 weeks consistent • Pain <3/10 • Feels "too easy"

How to Progress (Choose ONE)

Add reps

Add sets

Add resistance

Add hold time

Add complexity

Add frequency

When to Regress

Pain >4/10 that persists • Missing sessions •
Poor form despite cueing

Moelands et al. (2025): Higher prescribed frequency and dosage of home exercises significantly lowers patient adherence. Sometimes less is more.

QUICK REFERENCE 14

The Home Exercise Compliance Tracker

Photocopy this page. Fill it in each week. Bring it to your appointments.

Week of: _____

Exercise	Mon	Tue	Wed	Thu	Fri	Sat	Sun
1.	☐	☐	☐	☐	☐	☐	☐
2.	☐	☐	☐	☐	☐	☐	☐
3.	☐	☐	☐	☐	☐	☐	☐
4.	☐	☐	☐	☐	☐	☐	☐

Pain Level This Week (0-10): _____

Notes:

Check the box on days you did your exercises. Write your pain level at the end of the week. Bring this to your next appointment.

Documentation Phrases That Protect You

INSTEAD OF...	WRITE...
"Patient non-compliant."	"Patient reports difficulty adhering to HEP. Barriers discussed. Plan modified."
"Patient refused."	"Patient declined [intervention] at this time. Risks discussed. Alternative offered."
"Patient is fine."	"Patient reports no change in symptoms. Objective measures show [findings]."
"Patient was lazy."	"Patient demonstrated limited engagement today. Motivational strategies used."

"Treatment did not work."

"Current intervention yielded limited progress. Plan reassessed and modified."

"Patient needs to try harder."

"Patient would benefit from increased adherence to HEP. Support strategies discussed."

"No pain."

"Patient denies pain with reported activities. Pain-free ROM demonstrated."

"Patient is making excuses."

"Patient identified barriers to participation. Problem-solving approach used."

"Normal."

"Within functional limits. [Specific objective findings documented.]"

"I told the patient..."

"Patient educated on [topic]. Verbalized understanding. Questions answered."

PATIENT HANDOUT 1

Recovery Is Not a Straight Line

The Recovery Curve



You expect a straight line up. It never is.

4 Key Messages

1. Setbacks are normal. They are not starting over.

2. Progress hides in bad weeks. You are still moving forward even when it does not feel like it.

3. Pain spikes do not mean new damage. They mean your body is still calibrating.

4. The people who recover fastest are not the ones who never struggle. They are the ones who keep going when they do.

When to Call Your PT

Call if: Pain suddenly gets much worse and stays worse for 2+ days • You notice new numbness or tingling • You cannot bear weight • Something just feels wrong

PATIENT HANDOUT 2

Your Home Exercise Plan

Fill in the details for each exercise your PT gives you.

Exercise 1

Name: Reps: Sets: Frequency:

Notes:

Exercise 2

Name: Reps: Sets: Frequency:

Notes:

Exercise 3

Name: Reps: Sets: Frequency:

Notes:

Exercise 4

Name: Reps: Sets: Frequency:

Notes:

Week 1 Tracking

[illegible]



PATIENT HANDOUT 3

Pain vs. Damage: What You Need to Know

4 Key Facts About Pain

1. Pain is a protection signal, not a damage meter. It tells your brain something might be wrong — not that something IS wrong.

2. After an injury, your pain system becomes extra sensitive. It rings the alarm for things that are not actually dangerous.

3. Movement helps recalibrate this system. Avoidance makes it worse.

4. Soreness during exercise that settles within 24 hours is normal. Sharp, worsening, or persistent pain is not.

When Pain Is Normal

- Mild soreness during exercise that goes away
- Muscle ache the next day (DOMS)
- Discomfort in stiff joints that improves with movement
- Fatigue in weak muscles

When to Call Us

Call your PT if:

- Pain suddenly gets much worse and does not settle
- You notice new numbness, tingling, or weakness
- You cannot bear weight on the injured area
- Pain wakes you from sleep consistently

PATIENT HANDOUT 4

5 Things to Remember on Hard Days

- 1.** A hard day is not a bad day. It is just a hard day. Tomorrow is a new one.
- 2.** You do not have to be perfect. You just have to show up. Even a little counts.

3. Your body is not your enemy. It is doing its best with what it has. Work with it, not against it.

4. Progress is invisible on hard days. But it is still happening. Trust the process.

5. You are not alone in this. Every patient has hard days. The ones who recover are the ones who keep going anyway.

Cut this out. Keep it in your wallet. Read it on the hard days.

PATIENT HANDOUT 5

Questions to Ask Your Physical Therapist

You have the right to ask questions. Here are the important ones.

☐ What is my diagnosis in plain language?

☐ How long should recovery take?

☐ What should I do if my pain gets worse?

☐ What does "pain" during exercise mean? When should I push through and when should I stop?

☐ How many days per week should I do my exercises?

☐ What counts as "progress" and how will we measure it?

☐ What happens if this plan does not work? What is Plan B?

☐ Are there things I should avoid while I recover?

☐ Can I still do [activity I love]? If not, when?

☐ What can I do on my own, and what do I need you for?

BURNOUT MINI-TOOL 1

The Compassion Fatigue Check-In

Be honest. This is just for you.

Answer Yes or No:

- | | |
|---|-------|
| 1. I dread going to work most days. | Y / N |
| 2. I feel emotionally numb during patient sessions. | Y / N |
| 3. I take patient frustration home with me. | Y / N |
| 4. I have stopped caring about outcomes. | Y / N |
| 5. I feel like nothing I do makes a difference. | Y / N |
| 6. I use food, alcohol, or screens to cope. | Y / N |
| 7. I have frequent headaches, tension, or insomnia. | Y / N |
| 8. I have thought about leaving the profession. | Y / N |

Scoring

0-2 Yes: You are managing. Keep protecting your boundaries.

3-4 Yes: Warning signs. Time to make changes. Start with Quick Ref 8 (morning check-in) and Mini-Tool 2 (5-minute reset).

5+ Yes: Compassion fatigue is real. Talk to someone. A counselor, a mentor, or a peer. This is not weakness — it is wisdom.

Next Steps

If you scored 3+: Pick ONE thing from this list and do it this week: Take a real lunch break. Leave on time one day. Say no to one extra patient. Do the 5-Minute Reset before your first session. Talk to someone you trust.

BURNOUT MINI-TOOL 2

The 5-Minute Reset

Between patients. In your car. In the bathroom.
Wherever you can find 5 minutes.

Minutes 1-2: Breathing

*Breathe in for 4 counts. Hold for 4 counts.
Breathe out for 6 counts. Repeat 4 times.*

*As you exhale, imagine tension leaving
your shoulders, jaw, and hands.*

Minutes 3-4: Body Scan

*Close your eyes. Scan from head to toe.
Where are you tight?*

Forehead? Unclench it.

Jaw? Drop it.

Shoulders? Let them fall.

Stomach? Release it.

Hands? Open them.

Minute 5: Intention Setting

Pick ONE sentence and say it silently:

"I am here. I am enough. I will help who I can."

"I cannot fix everything. But I can be present for this next person."

"I give myself permission to not be perfect."

BURNOUT MINI-TOOL 3

Your Boundary Script Kit

5 scripts for saying no — without guilt.

1. Scheduling

"I cannot add another patient today. I know that is frustrating. Let us find you the earliest available time this week."

2. Documentation

"I need to finish my notes before I leave today. I cannot stay late again. I will prioritize documentation between sessions."

3. Taking Lunch

"I am taking my lunch break today. I will be available again at [time]. If it is urgent, [colleague] can help."

4. Leaving on Time

"I have a hard stop at [time] today. I want to make sure we cover the most important things first."

5. Emotional Boundaries

"I care about what you are going through. I also need to be honest about what I can help with. Let us focus on what is in my scope, and I will help you find support for the rest."

CUSTOM TEMPLATE

Build Your Own Script

Fill in the blanks. Create a script for any situation.

Situation

Describe the patient situation here...

What Not to Say

What is the instinctive response that does not work?

Why does it not work?

Option 1

First approach — what to say...

Option 2

Second approach — what to say...

Option 3

Third approach — what to say...

Why This Works

Explain the mechanism — why this approach helps...

Research Backing

Cite the evidence or theory that supports this approach...

YOUR DAILY COMPANION

This Desk Kit Is Your Daily Companion

But it is only the beginning.

The **PT Communication Resource Core Bundle** includes everything in this desk kit plus:

- 25** Core Explanations
- 35** Session Scripts (including all 5 from the free resource, expanded)
- 30** Q&As — real questions from PTs, answered with research
- 12** Companion Tools (templates, reflection logs, checklists)
- 12** Full Burnout Protection Tools
- +** AI-Powered PTComm App

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